INSIGHTS CORNER



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What is insights Corner?

INSIGHTS CORNER

EULAR'25-Annual European Congress of Rheumatology (11-14 Jun 2025)

Objectives:

- Present the latest clinical and research advancements across the full spectrum of rheumatic and musculoskeletal diseases
- Provide educational sessions and hands-on workshops to improve clinical practice and patient care
- Encourage the submission and presentation of high-quality research, and support collaboration through networks like the EULAR Network of Trial centres (ENTRI)

Clinical Status	No. of Sessions	No. of Presentations	No. of Faculty
Pre-Clinical	No specified Sessions	4	
Phase 1		8	
Phase 1/2		1	
Phase 2		32	1
Phase 2/3		1	1
Phase 3		36	1
Phase 4		3	

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Psoriatic Arthritis in EULAR 2025

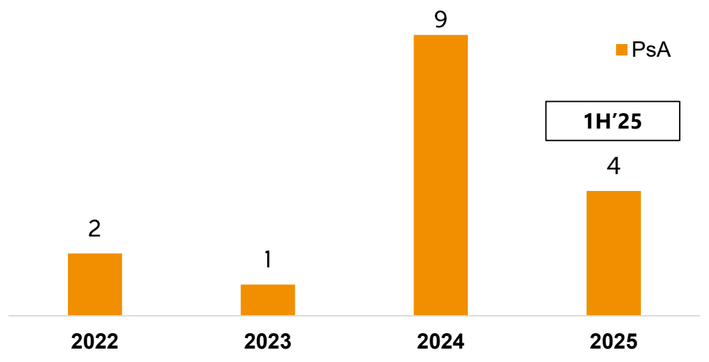
- 2025 EULAR recommends early, intensive treatment for PsA, especially in high-risk patients
- The SPEED trial found early TNF inhibitor therapy gives lasting benefits at 48 weeks, unlike combination DMARDs
- Early data shows combining biologics and targeted synthetic DMARDs is effective and safe short-term, but long-term safety needs more study
- Insights of EULAR 2025:
  - Long-Term Efficacy of Bimekizumab (BIMZELX)
    - ✓ Three-year data showed sustained control of inflammation and symptom relief in psoriatic arthritis (PsA) patients
    - ✓ Over half of patients maintained stringent ACR50 responses at three years, with consistent safety and no new concerns
    - ✓ Most common adverse events were mild, including COVID-19 infection, nasopharyngitis, and upper respiratory tract infections
  - Treat-to-Target (T2T) in Real-World Practice
    - ✓ The MONITOR-PsA study confirmed that a T2T approach is feasible in routine care
    - ✓ By 48wks, 37% of patients achieved a 'good' response (PASDAS), 47.6% reached minimal disease activity and 32.7% achieved DAPSA remission
    - ✓ Nearly 70% of patients reported an acceptable symptom state, and no structural damage progression was observed during the study period
  - Emerging Therapies
    - ✓ Sonelokimab, a Nanobody targeting IL-17A and IL-17F, demonstrated strong joint and skin responses in the ARGO P2 trial, with high rates of minimal disease activity and favorable safety
    - ✓ Deucravacitinib, an oral TYK2 inhibitor, showed clinical benefits over placebo at 16 weeks, adding to the expanding therapeutic options
  - Personalized Medicine and Biomarkers
    - ✓ EULAR 2025 highlighted the growing role of biomarkers and precision medicine, including risk stratification and synovial fluid analysis, to tailor therapy and predict outcomes in PsA
  - Digital and AI-Driven Care
    - ✓ Sessions emphasized the integration of artificial intelligence and digital tools for disease monitoring, risk prediction, and optimizing treatment selection

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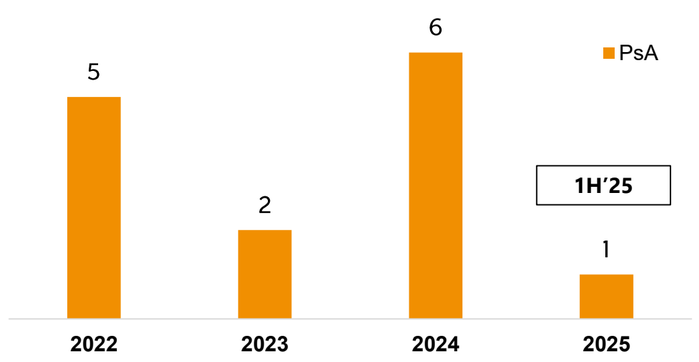
Indication Focus: Psoriatic Arthritis

Market Overview of PsA

Last 4 years deals in PsA



Last 4 years funding in PsA



In 2025

Deals

Sales/ Co-Promotion

SAMSUNG BIOEPIS — NIPRO

百奥泰 BIO-THERA — Dr.Reddy's

accord BioPharma

Platform, Product Purchase

teva — Lumicera Health Services

Funding

MoonLake

$75M loan from

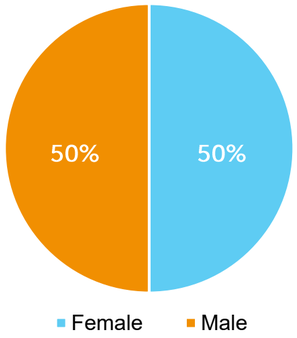
Hercules CAPITAL

PsA Epidemiology

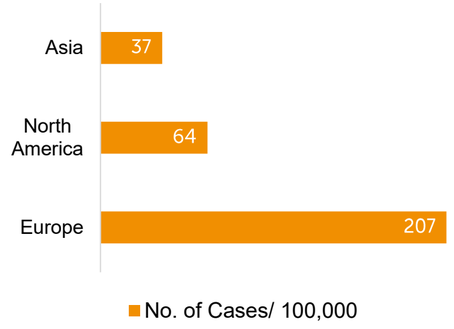
Psoriatic Arthritis Epidemiology in Europe (to 2025):

- **Prevalence:** 207 per 100,000 adults (range: 113–379 per 100,000), with country-specific rates from 51 up to 549 per 100,000
- **Incidence (Sweden):** 20–22 new cases per 100,000 adults/year (2014–2016)
- **Regional Comparison:** Europe has the highest prevalence globally (207 per 100,000), compared to North America (64) and Asia (37)
- **Age at Onset:** Most cases begin between 30 and 60 years; both men and women are equally affected

Gender Distribution



Region Comparison



Highlights

2025 UC Davis study

- Total-Body PET scanner to detect inflammation in PsA patients more accurately than standard exams
- This technology finds inflammation in joints, entheses, and nails, enabling earlier diagnosis and better treatment decisions
- It marks a major advancement in assessing and managing Psoriatic Arthritis

OMERACT framework

- The article reviews the latest ways to assess psoriatic arthritis, helping clinicians choose the best tools
- GRAPPA has identified key areas to measure in PsA and is working on the best methods, following the OMERACT framework
- New composite measures are being developed for a more complete and efficient evaluation of PsA

Abb.: PsA-Psoriatic Arthritis, RA- Rheumatoid arthritis, 1H'25- First half of 2025

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Indication Focus: Psoriatic Arthritis

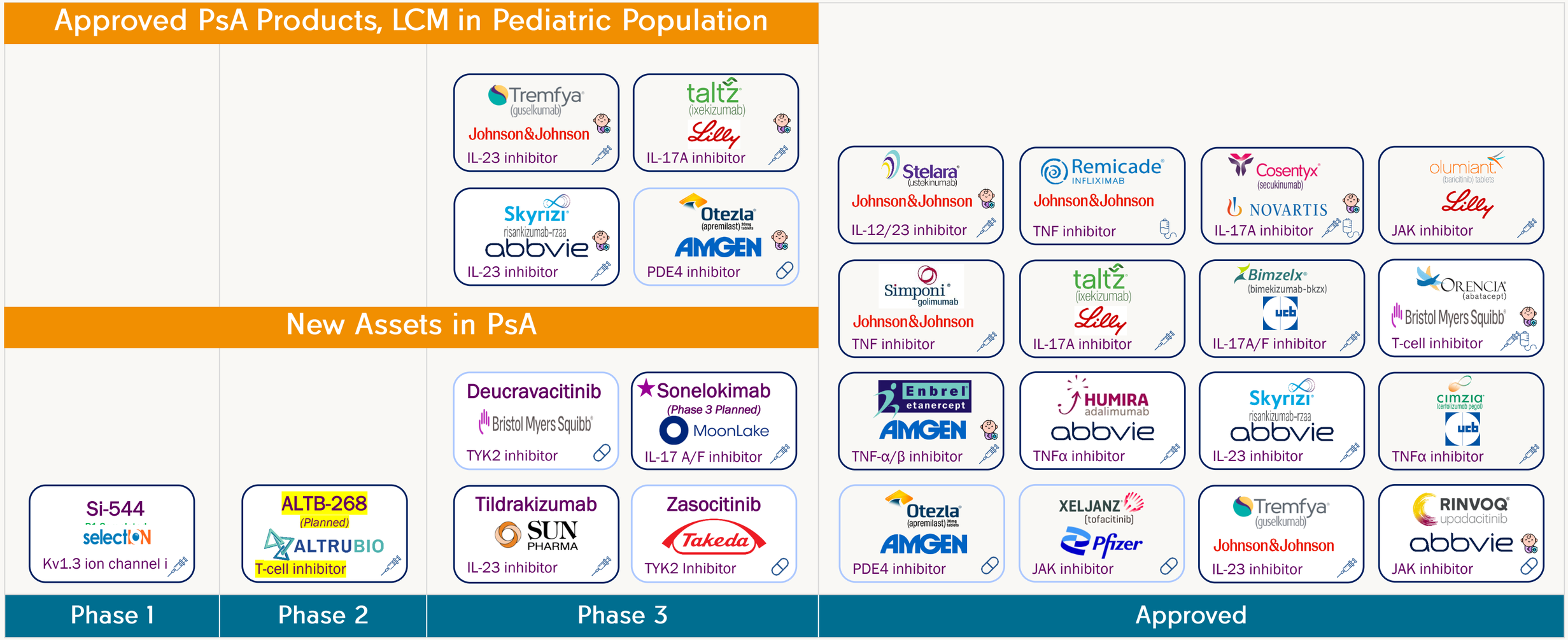
Deals Analysis

Date	Companies	Deal Type	Asset	Country
06-08-2025	Samsung Bioepis – Nipro Corp.	Sales / Co-Promotion	SB-17	South Korea – Japan
03-27-2025	Bio-Thera – Dr. Reddy's Laboratories	Sales / Co-Promotion	BAT-2206, BAT-2506	China – India
03-13-2025	Teva – Lumicera Health Services (Navitus Health Solutions)	Platform, Product Purchase	Stelara (ustekinumab)	Israel – USA
02-10-2025	Bio-Thera – Accord BioPharma	Sales / Co-Promotion	BAT-2506	China – USA
10-09-2024	Bio-Thera – Gedeon Richter	Development & Commercialization	BAT-2206	China – Hungary
07-26-2024	Nordic Pharma – Magnapharm Holdings	Sales / Co-Promotion	Nordimet	USA – Romania
06-27-2024	Hong Kong King-Friend Industrial – Meitheal Pharmaceuticals	Sales / Co-Promotion	Yusimry, biosimilars for pegfilgrastim, filgrastim, and follitropin alpha	Hong Kong – USA
05-28-2024	Bio-Thera – Stada Arzneimittel	Development & Commercialization	BAT-2506	China – Germany
05-27-2024	Clarius – Novartis	Contract Service	Handheld ultrasound device	Canada – Switzerland
04-23-2024	AndHealth – PrimaryOne Health	Contract Service	In-house specialty care service	USA – USA
04-19-2024	Alvotech – Teva	Sales / Co-Promotion	Simlandi (adalimumab-ryvk)	Iceland – Israel
04-11-2024	Sandoz – Lupin	Contract Service	Rymti	Germany – India
03-28-2024	Paragon Therapeutics – Oruka Therapeutics	Option to License	ORKA-001, ORKA-002	USA – USA
12-06-2023	IMIDomics – Bristol Myers Squibb	Contract Service	Clinical Discovery Engine	USA – USA
12-17-2022	Biocon – Yoshindo	Development & Commercialization	Ustekinumab and Denosumab	India – Japan
04-04-2022	Microba Life Sciences – Ginkgo Bioworks	R&D Only	Microba's biobank with Ginkgo's screening capabilities	Australia– USA

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Indication Focus: Psoriatic Arthritis

Clinical Landscape (EU)



Oral · Intravenous · Intra-articular · Subcutaneous · Paediatric · Adults · Biologic · Small Molecule

Note: Assets without pediatric icon represent adult population

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Indication Focus: Psoriatic Arthritis

Biosimilar Landscape (EU)

SIMPONI /SIMPONI ARIA

Phase 3

- BAT2506 — 百奥泰 BIO-THERA / STADA — TNF-α inhibitor
- AVT05 — alvotech — TNF-α inhibitor

COSENTYX

Phase 3

- CT-P55 — HEALTHCARE CELLTRION — IL-17A inhibitor
- BAT2306 — 百奥泰 BIO-THERA — IL-17A inhibitor
- DMB-3115 — accord — IL-12/23 inhibitor
- BAT2206 — 百奥泰 BIO-THERA — IL-12/23 inhibitor
- FYB202 — FRESENIUS KABI / formycon — IL-12/23 inhibitor

REMICADE

Approved

- Remsima — Infliximab — HEALTHCARE CELLTRION
- Inflectra — infliximab-dyyb — HEALTHCARE CELLTRION
- Zessly — SANDOZ
- Flixabi — infliximab — SAMSUNG BIOEPIS

HUMIRA

Approved

- Yuflyma — adalimumab-aaty — HEALTHCARE CELLTRION
- Imraldi — Adalimumab — SAMSUNG BIOEPIS
- AMGEVITA (adalimumab) — AMGEN BIOSIMILARS
- Hulio — adalimumab injection — BCIL
- Hyrimoz — adalimumab — SANDOZ
- Hukyndra — adalimumab — STADA
- Libmyris — STADA
- Idacio — adalimumab — FRESENIUS KABI
- Amsparity — Pfizer
- Hefiya — adalimumab — SANDOZ

STELARA

Approved

- Yesintek (ustekinumab-kfce) — BCIL
- WEZENLA — AMGEN BIOSIMILARS
- QOYVOLMA — HEALTHCARE CELLTRION
- IMULDOSA — accord — Make it better
- UZPRUVO — ustekinumab — STADA
- Pyzchiva — ustekinumab-ttwe | 90 mg/1 mL — SAMSUNG BIOEPIS
- OTULFI — FRESENIUS KABI
- Steqeyma — HEALTHCARE CELLTRION
- EKSUNBI — SAMSUNG BIOEPIS

ENBREL

Approved

- Benepali — Etanercept — SAMSUNG BIOEPIS
- Erelzi — (etanercept-szzs) — SANDOZ
- Nepexto — BCIL

Intravenous | Subcutaneous | Paediatric | Adults

BCIL: Viatris & Biocon

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Product Dashboard: Bimzelx

Bimzelx® (bimekizumab-bkzx)

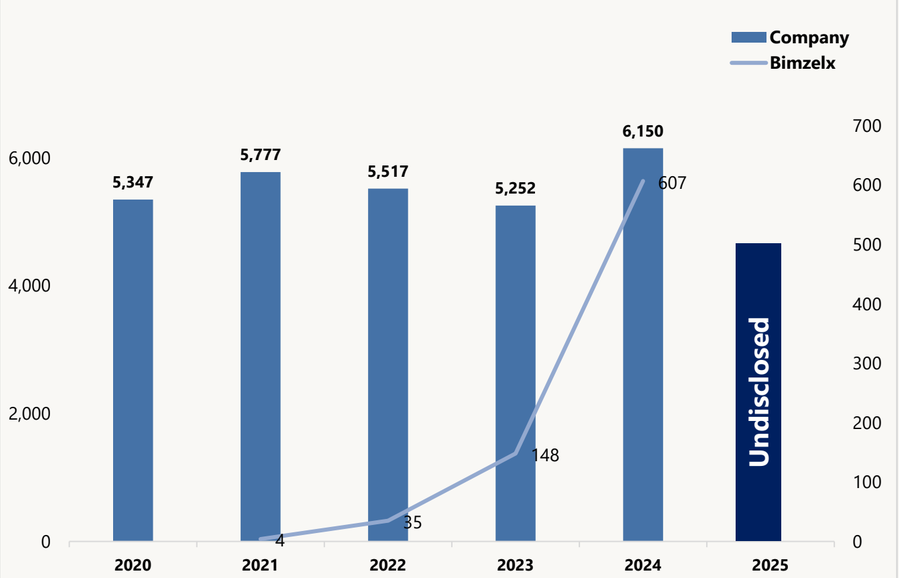
COMPANY OVERVIEW

Headquarter	Belgium
Therapy area focus	Neurology, Immunology
Total Revenue in 2024	€ 6.15B ($7.22B)
R&D Spend in 2024	€ 1,781M ($2,090.54M)

PRODUCT OVERVIEW

MoA	Humanized interleukin-17A and F antagonist
Dosage	160 mg/mL (single-dose prefilled syringe or single-dose prefilled autoinjector)
Formulation	Subcutaneous
Initial Approval	2021

SALES



EU APPROVAL TIMELINE

Aug'21 — Approved for Moderate to Severe Plaque Psoriasis

Jun'23 — Approved for **PsA**, Non-Radiographic Axial Spondyloarthritis

Apr'24 — Approved for Moderate to Severe Hidradenitis Suppurativa

US APPROVAL TIMELINE

Oct'23 — Approved for Moderate to Severe Plaque Psoriasis

Sep'24 — Approved for **PsA**, Non-Radiographic Axial Spondyloarthritis and Ankylosing Spondylitis

Oct'24 — Approved for 320 mg Single-Injection Device

Nov'24 — Approved for Moderate to Severe Hidradenitis Suppurativa

HIGHLIGHT

Bimzelx represents a significant advance in the management of psoriatic arthritis, plaque psoriasis, and related conditions, with robust efficacy, a favorable safety profile, and long-term benefits for diverse patient populations

Exchange Rate: €1 = $1.1738

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BE COMPLETE & BE OPTIMAL (P-III) Study Results

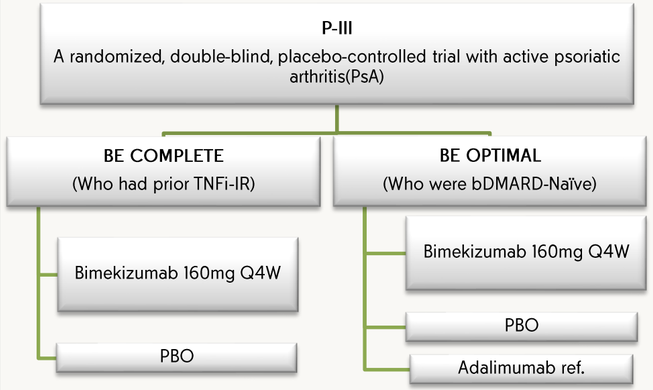
BIMEKIZUMAB

(a monoclonal IgG1 antibody that selectively inhibits interleukin (IL)-17F in addition to IL-17A)

ABOUT STUDY

BE COMPLETE (NCT03896581), BE OPTIMAL (NCT03895203)
Long-term safety and efficacy of Bimekizumab (BKZ) given 160mg with Q4W, total group include, BKZ randomized patients and PBO patients that switched to BKZ @16wks

STUDY DESIGN



EFFICACY

Efficacy Endpoints	Bimekizumab 160mg Q4Wz			
	BE COMPLETE (N=400) @156wks		BE OPTIMAL (N=712) @160wks	
	mNRI	OC	mNRI	OC
ACR 20 response (%)	71.7	85.6	67.6	80.1
ACR 50 response (%)	37.4	47.6	40.0	50.1
ACR 70 response (%)	84.6	96.5	84.2	93.8
PASI 75 response (%)	77.3	89.5	76.4	88.0
ACR 70 + PASI 100 response (%)	48.3	60.5	44.1	54.4

SAFETY

- Few patients stopped treatment because of Candida infections
- Only one serious case of oropharyngeal candidiasis was reported over three years

IMPLICATIONS

- BKZ maintained IL-23 pathway inhibition for 3 years, achieving sustained ACR50 (55.2%) and PASI100 (67.5%) response rates
- The treatment had a favorable safety profile, with few major adverse events (5.7 per 100 patient-years) and manageable Candida infections (5.8 per 100 patient-years) and most common adverse events were COVID-19, nasopharyngitis, and upper respiratory tract infections, with very few cardiovascular events reported
- Bimekizumab was generally well tolerated, and no new safety concerns emerged

Abb.: ACR-American College of Rheumatology response criteria, PBO-Placebo, BKZ-Bimekizumab, mNRI-modified non-responder imputation, OC-Observed case

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Product Dashboard: Sonelokimab

MoonLake

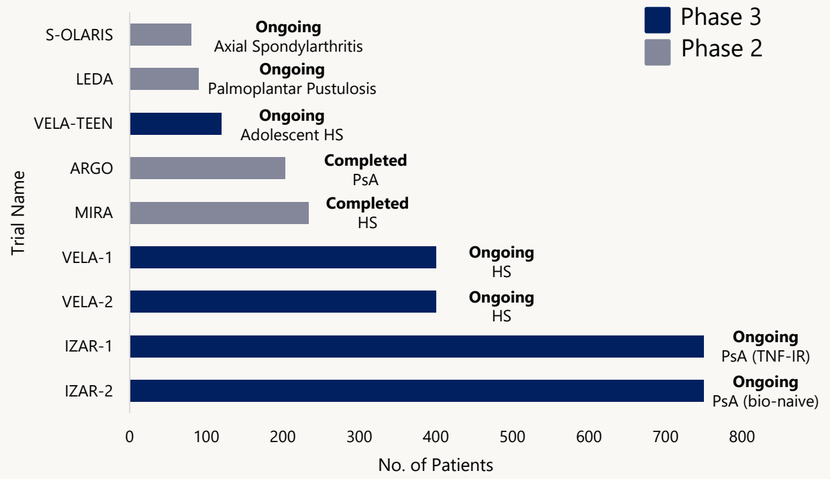
PRODUCT OVERVIEW

Sonelokimab represents a potentially transformative therapeutic asset in the inflammatory disease space, currently advancing through multiple P3 trials

Originally discovered by **Ablynx** and now been developed by **MoonLake Thera** with strategic partnerships, this innovative nanobody targets both **IL-17A** and **IL-17F** pathways, offering a differentiated approach compared to existing IL-17 inhibitors

- ✓ Pivotal data expected in **Sep'2025**
- ✓ Commercial launch anticipated in **2027**

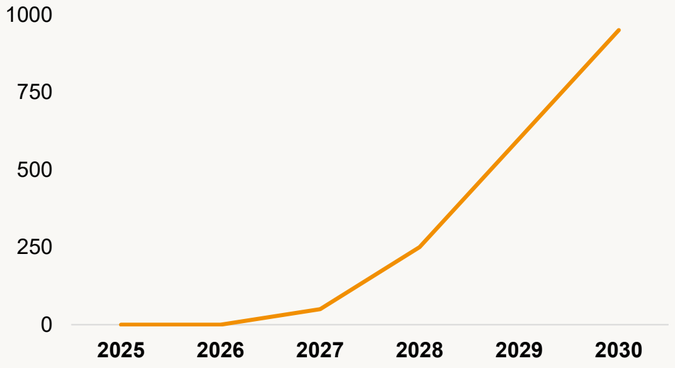
SONELOKIMAB CT PORTFOLIO



HISTORICAL DEAL STRUCTURE OVERVIEW

Year	Parties Involved	Key Details
2013	**Merck KGaA & Ablynx**	Merck KGaA acquired full, exclusive global rights; established initial commercial framework
2017–2020	**Merck KGaA & Avillion**	Co-development of Phase II trials; successful Phase IIb psoriasis trial exceeded secukinumab
2021	**MoonLake & Merck KGaA**	MoonLake in-licensed asset; upfront (low double-digit €m), ~10% equity, milestones, royalties
2023	**MoonLake & SHL Medical**	Autoinjector partnership (Molly platform); covers clinical/commercial supply; global reach
2025	**MoonLake & Hercules Capital**	$500m non-dilutive debt facility; $75m drawn at closing; further tranches tied to milestones

REVENUE PROJECTION ($M)



FINANCIAL POSITION AND REVENUE PROJECTION

- ✓ **MoonLake** holds $980.1M in total capital (including Hercules facility), maintains disciplined quarterly R&D spend of $36.5 million, and is funded through 2028 for all pivotal readouts
- ✓ **Phase 3** data and regulatory submissions (2025-2026) are expected to establish sonelokimab's commercial viability and support initial market entry
- ✓ Analysts project a phased commercial launch with $50 million revenue in **2027**, expanding to $600 million in **2029**, and reaching peak global sales of $949 million by **2030**

COMPETITIVE ANALYSIS

- ✓ **Sonelokimab** enters a competitive but rapidly expanding IL-17 inhibitor market valued at $27.3B in 2024, projected to reach $85.2B by 2030 (17.3% CAGR)
- ✓ **Established Competitors:**
  - **Cosentyx (secukinumab):** Market leader with >40% share in new rheumatology prescriptions
  - **Taltz (ixekizumab):** Strong competitor with high psoriasis efficacy
  - **Bimzelx (bimekizumab):** New entrant with dual IL-17A/F inhibition
- ✓ **Sonelokimab's** unique tri-specific nanobody enables superior tissue penetration and best-in-class potential across multiple indications

Exchange Rate: €1 = $1.1738

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MoonLake

ARGO (P-II) Study Results
SONELOKIMAB

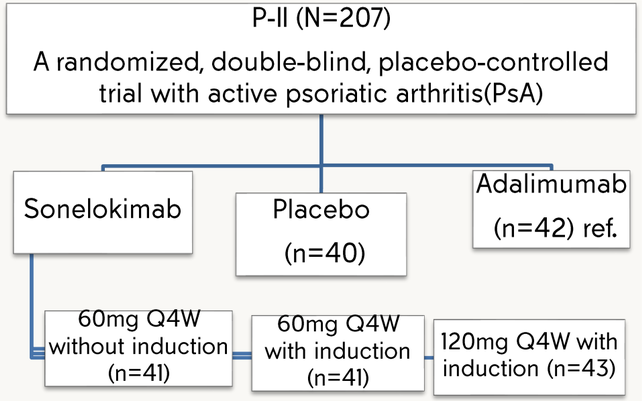
(an IL-17A- and IL-17F-inhibiting Nanobody)

ABOUT STUDY

ARGO (NCT05640245)

Patients were randomly into 5 groups: sonelokimab Q4W: 120 mg with induction, 60 mg with induction, 60 mg without induction, placebo group or adalimumab 40 mg Q2W. The first 12wks were placebo-controlled, after which placebo patients switched to sonelokimab 120 mg without induction.

STUDY DESIGN



SAFETY

- Sonelokimab was well tolerated, and no unexpected safety issues were observed

EFFICACY

Measures		Sonelokimab 60 mg without induction	Sonelokimab 60 mg with induction	Sonelokimab 120 mg with induction	PBO	Adalimumab (ref)
LSM change from baseline @12wks	PsAID-12 P value	-2.5 <0.001	-2.3 0.001	-2.2 0.002	-1.3 -	-2.2 -
	PtAAP P value	-30.4 0.011	-30.9 0.008	-30.1 0.012	-18.8 -	-29.5 -
	PGA P value	-31.9 0.003	-28 0.034	-27.8 0.038	-18.7 -	-29.2 -
Outcomes @24wks	MDA (%)	46.3	61	51.2		45.2
	ACR 70 + PASI 100 (%)	31.3	34.6	48.1		18.8
	ACR 70 (%)	39	39	41.9		35.7
	PASI 100 (%)	59.4	61.5	63		50

IMPLICATIONS

- By 24wks, most patients achieved remission with Sonelokimab, showing significant improvements in symptoms, discomfort, and quality of life
- Patient-reported outcomes highlight Sonelokimab's strong potential as a comprehensive PsA therapy, pending phase 3 confirmation

Abb.: ACR-American College of Rheumatology response criteria, PBO-Placebo, MDA-Minimal disease activity

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Bristol Myers Squibb

POETYK PsA-1(P-III) Study Result

DEUCRAVACITINIB

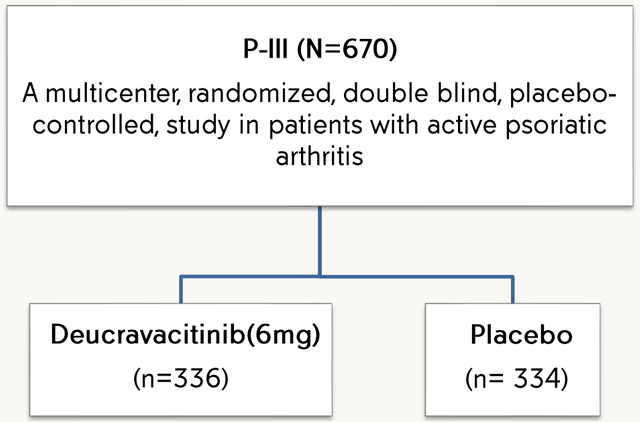
(first-in-class, oral, selective, allosteric tyrosine kinase 2 (TYK2) inhibitor)

ABOUT STUDY

POETYK PsA-1 (NCT04908202)

A diagnosis of PsA for at least 3 months is required, meeting the CASPAR criteria. Patients must have active or documented psoriasis, active arthritis with three or more swollen and tender joints, a high-sensitivity C-reactive protein (hsCRP) level of 3 mg/L or higher, and at least one joint erosion in the hands or feet related to PsA

STUDY DESIGN



EFFICACY

Efficacy endpoints @16wks			Deucravacitinib 6 mg QD	PBO	P value
Primary Endpoints	Composite clinical efficacy	ACR 20 response (%)	54.2	34.1	<0.0001
		ACR 50 response (%)	24.7	13.5	0.0002
		ACR 70 response (%)	11.6	5.4	0.0039
Secondary Endpoints		PASI 75 response (%)	51.9	7.1	<0.0001
		MDA response (%)	19.0	10.2	0.0012
	Patient-reported outcomes	HAQ-DI score, mean CfB	-0.39	-0.22	<0.0001
		SF-36 PCS score, mean CfB	6.06	3.71	<0.0001

SAFETY

- Due to AE 2.4% discontinued
- Upper respiratory tract infection (5.1%)
- Death 0%

IMPLICATIONS

- Deucravacitinib improved joint, skin, and QoL outcomes by Week 16 with less radiographic progression
- Safety was consistent; no new risks, mild AEs only

Abb.: ACR-American College of Rheumatology response criteria, CfB-Change from baseline, PBO-Placebo, MDA-Minimal disease activity

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Bristol Myers Squibb

POETYK PsA-2 (P-III) Study Result

DEUCRAVACITINIB

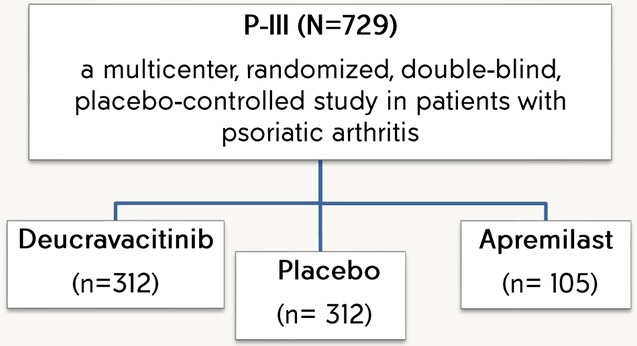
(first-in-class, oral, selective, allosteric tyrosine kinase 2 (TYK2) inhibitor)

ABOUT STUDY

POETYK PsA-2 (NCT04908189)

Mean disease duration since diagnosis was 5.35 years (range: 0.2–39.8 years). At baseline, 30.2% of patients had an hsCRP level of ⩾ 10 mg/L, 62.8% were on conventional synthetic disease-modifying antirheumatic drugs, and 13.4% had previously received a TNF inhibitor

STUDY DESIGN



SAFETY (0-52wks)

- Due to AE 2.2% discontinued
- Nasopharyngitis (11.8%), COVID-19 (11.6%), Upper respiratory tract infection (10.9%)

EFFICACY

Efficacy endpoints			Deucravacitinib 6 mg QD	PBO	PBO to Deucravacitinib 6 mg QD	Apremilast 30 mg BID	P value
Primary Endpoints	ACR Response @16wks	ACR 20 response (%)	54.2	39.4	-	41.9	0.0002
		ACR 50 response (%)	28.8	16.3	-	19.1	0.0002
		ACR 70 response (%)	10.6	5.4	-	6.7	0.0180
	ACR Response @52wks	ACR 20 response (%)	62.2	-	67.3	52.4	-
		ACR 50 response (%)	40.7	-	43	33.3	-
		ACR 70 response (%)	26	-	27.6	15.2	-
Secondary Endpoints	-	PASI 75 response (%)	40.9	15.4	-	-	<0.0001
		MDA response (%)	25.6	14.7	-	-	0.0007

IMPLICATIONS

- @16wks, Deucravacitinib delivered robust efficacy, achieving ACR 20 in 54.2% and PASI 75 in 40.9% of patients versus PBO
- Through 52wks, Deucravacitinib maintained a favorable safety and durability profile, with low serious AE (1.9%) and discontinuation rates (2.2%) and no excess CV, infectious, or malignancy risks

Abb.: ACR-American College of Rheumatology response criteria, CV-Cardiovascular, PBO-Placebo, MDA-Minimal disease activity

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Johnson&Johnson

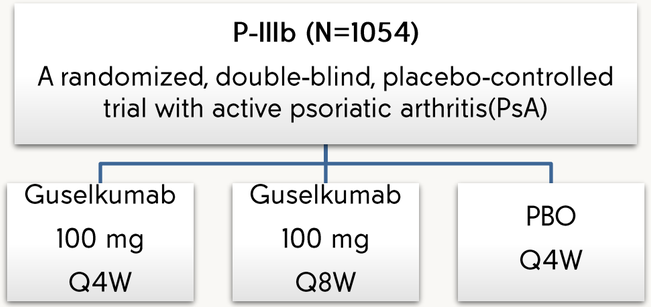
APEX (P-IIIb) Study Results
GUSELKUMAB

(monoclonal antibody that works by blocking a protein called interleukin-23 (IL-23))

ABOUT STUDY

APEX (NCT04882098)
Guselkumab (GUS) given 100mg with Q4W & Q8W, Results from a Randomized, Double-Blind, Placebo-Controlled Study Through @24wks

STUDY DESIGN



SAFETY @24wks

- Adverse Events: 38%, 42%, and 37% of pts (Guselkumab 100 mg Q4W or Q8W and PBO Q4W respectively) mainly respiratory infections, headache, diarrhea, psoriatic arthropathy
- Serious Adverse Events: 2%, 3%, and 3% of pts, in the GUS Q4W, GUS Q8W, and PBO groups, respectively

EFFICACY

Efficacy endpoints @24wks			Guselkumab 100 mg		PBO (n=376)
			Q4W (n=273)	Q8W (n=371)	
Primary Endpoints	Clinical Efficacy	ACR 20 response (%)	66.6	68.3	47.0
		ACR 50 response (%)	41	42	20
		ACR 70 response (%)	22	22	11
		PASI 90 response (%)	69	60	22
Secondary Endpoints	Patient-reported outcomes	HAQ-DI score	-0.41	-0.42	-0.26
		IGA 0/1 Response (%)	73	68	31

IMPLICATIONS

- Significantly improved clinical outcomes in biologic-naïve adults with active psoriatic arthritis, with 67–68% achieving ACR20 responses versus 47% on placebo.
- Both 4-week and 8-week dosing regimens of GUS effectively slowed radiographic joint damage progression compared to placebo.
- Safety profiles were similar across all groups, with no new safety concerns identified.
- The APEX phase 3b trial is the first to demonstrate that both dosing schedules of guselkumab can significantly inhibit structural joint damage in biologic-naïve PsA patients while maintaining a favorable safety profile

Abb.: ACR-American College of Rheumatology response criteria, PBO-Placebo, pts: Patients

INSIGHTS CORNER

abbvie

SELECT - PsA 1 (P-III) Study Results

UPADACITINIB

(a JAK inhibitor, works by blocking the action of Janus kinases, enzymes that play a role in inflammation)

ABOUT STUDY

SELECT - PsA 1 (NCT03104400)
Long-term safety and efficacy of Upadacitinib 5 years result from P3 with patients with Psoriatic Arthritis @12wks

STUDY DESIGN

P-III

A randomized, double-blind, placebo-controlled trial with active psoriatic arthritis(PsA)

- Upadacitinib 15 mg QD
- Upadacitinib 30 mg QD
- Adalimumab 40 mg EOW

SAFETY

- Long-term treatment with Upadacitinib (UPA) for up to five years showed no new safety risks and had a safety profile similar to Adalimumab (ADA)

EFFICACY

Upadacitinib (UPA) vs Adalimumab (ADA) (260wks-5 Years)						
Endpoints	UPA 15mg		UPA 30mg		ADA 40 mg	
	AO	NRI	AO	NRI	AO	NRI
ACR 20 (%)	90.3	53.4	86.6	51.8	86.3	52.0
ACR50 (%)	71.8	44.1	71.2	42.8	66.4	39.4
ACR70 (%)	47.1	29.4	48.4	29.6	44.7	28.9
MDA (%)	58.9	33.8	62.5	36.6	55	34.0
PASI 75 (%)	78.4	44.9	79.8	47.1	85.2	37.4
PASI 90 (%)	67.2	38.8	65.9	38.6	71.9	32.7
PASI 100 (%)	47	27.6	52.7	31.0	51.1	22.7

IMPLICATIONS

- The P3 SELECT-PsA 1 study evaluated Upadacitinib (UPA) for psoriatic arthritis patients unresponsive to non-biologic DMARDs over five years
- UPA 15 mg demonstrated sustained efficacy comparable to adalimumab (ADA) in improving joint and skin symptoms, physical function, and limiting radiographic progression
- Safety profiles of UPA and ADA were generally similar, with low rates of serious events like malignancies, cardiovascular issues, and thromboembolism
- UPA was associated with higher rates of serious infections and herpes zoster compared to ADA, especially at the 30 mg dose
- Overall, the study supports the long-term use of UPA 15 mg as a safe and effective treatment for psoriatic arthritis

Abb.: ACR-American College of Rheumatology response criteria, pts: Patients, QD: Once a Day; EOW: Every Other Week

Offerings

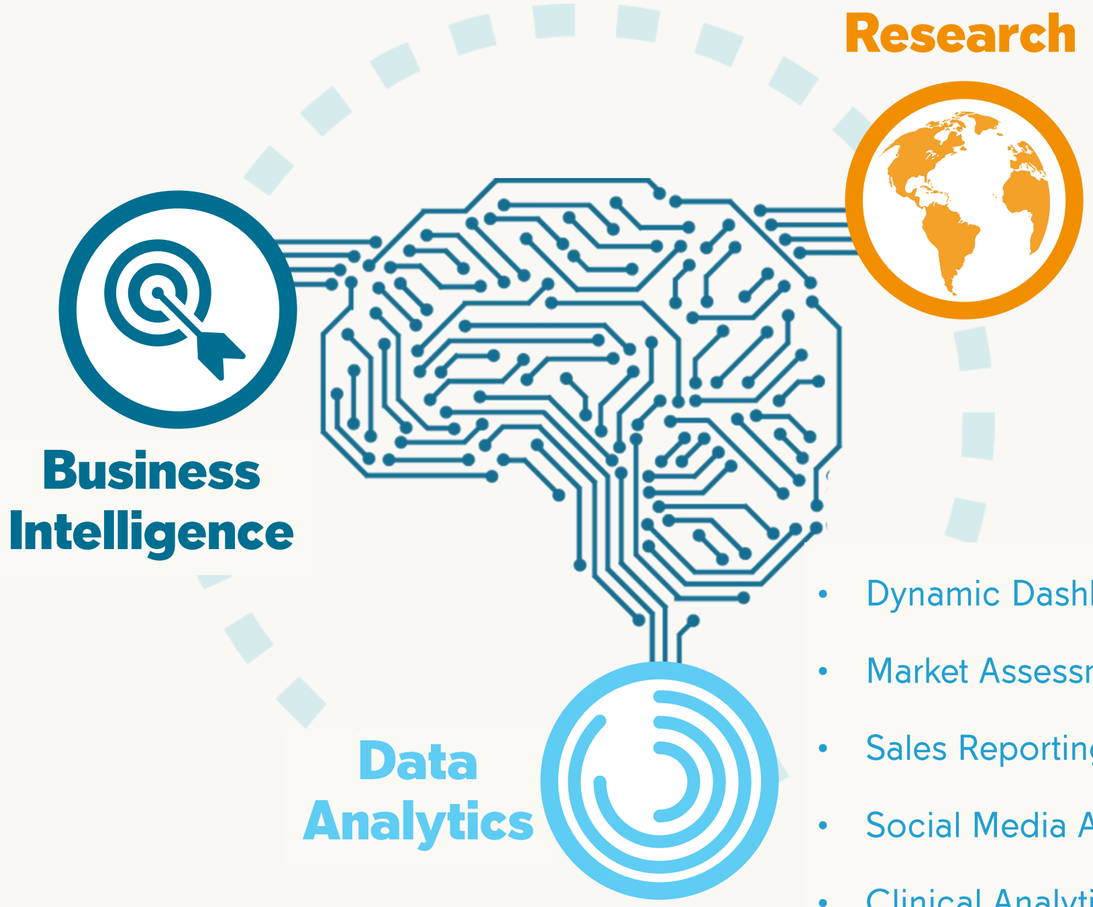
We specialize in delivering tailored consulting solutions that flexibly meet each client's unique needs, ensuring personalized and effective outcomes.

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- Awareness, Trial & Usage (ATU) Studies
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- Conference Coverage
- Market Sizing & Opportunity
- Pre-investment Due Diligence
- Company/Asset Prioritization



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- Market Assessment & Epidemiology Analysis
- Sales Reporting & In-line Forecasting
- Social Media Analytics
- Clinical Analytics

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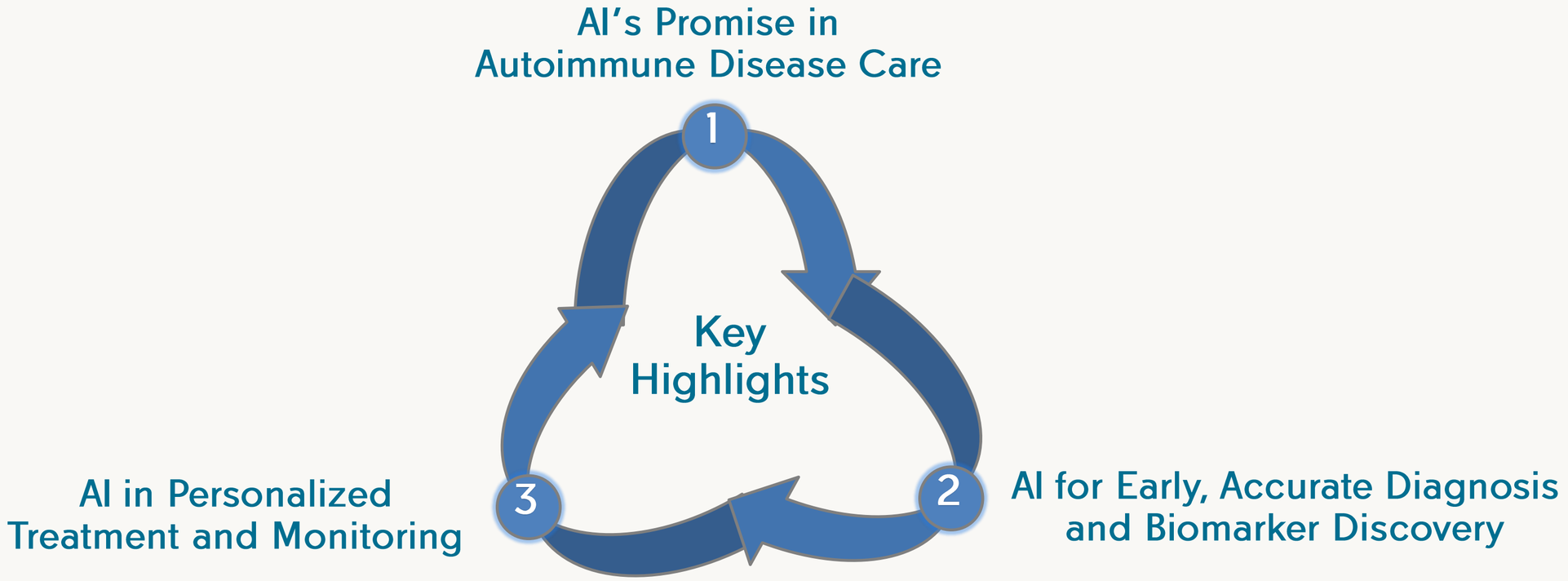
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INSIGHTS CORNER

Improving Autoimmune Disease Care With Artificial Intelligence



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